



## POLICY

# Increased Clinical Observation Levels in CAMHS Inpatient Services

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| <b>Scope (Staff):</b> | Clinical staff           |
| <b>Scope (Area):</b>  | CAMHS Inpatient Services |

### Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

## Contents

|                                                                                            |   |
|--------------------------------------------------------------------------------------------|---|
| Aim .....                                                                                  | 1 |
| Risk.....                                                                                  | 2 |
| Background.....                                                                            | 2 |
| Definitions .....                                                                          | 2 |
| Principles / Key points.....                                                               | 2 |
| Procedure .....                                                                            | 3 |
| Assessing clinical observation levels.....                                                 | 3 |
| Implementing clinical observation levels .....                                             | 3 |
| Reviewing clinical observation levels for patient on increased levels of observation ..... | 4 |
| Documentation required to assess clinical observation levels .....                         | 5 |
| Appendix 1: Increased Observation Levels .....                                             | 8 |

## Aim

To facilitate safe practice in determining a patient's clinical observation level by incorporating risk assessment, consultation, documentation and appropriate resource allocation of staff.

## Risk

- Failure to maintain adequate clinical observation levels may increase the risk of a sentinel event occurring, including patient suicide or critical injury.
- Failure to utilise resources appropriately may create unintended staffing shortfalls in other areas.

## Background

Risk to the child's safety is the key factor in determining where the patient is managed and how their care is coordinated.

## Definitions

**Special:** Provision of one-to-one care to a patient.

**Level of Observation:** Frequency of observations according to clinical or behavioural needs.

**Medical delegate:** On-call registrar under the supervision of the on-call consultant psychiatrist.

**Psychiatrist:** Refers to the treating Consultant Psychiatrist or in their absence, the on-call Consultant Psychiatrist.

## Principles / Key points

- When determining and initiating new clinical observation levels, clinicians must have regard to the following principles of care:
  - To provide patient centred care.
  - Using the least restrictive alternative required to effectively manage the current risk.
  - Ensuring safety and managing clinical risk.
  - To consider the cultural and religious needs of the young person.
  - To enable appropriate use of staffing resources and efficient use of financial resources.
  - Whenever possible alternative strategies to address patient needs should be explored.
- Level 4 general observations, which are done hourly, are the minimum acceptable level of observation for all patients.

## Procedure

### Assessing clinical observation levels

| Steps                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     | Additional Information                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <p>1. Clinical observation levels must be determined:</p> <ul style="list-style-type: none"> <li>• At preadmission where information is available</li> <li>• On admission</li> <li>• When there is information to indicate that the child is at risk in the Child Risk Assessment and Management Plan (CRAMP) or the Progressive Risk Assessment (PRA) and /or</li> <li>• By the Psychiatrist (or medical delegate) who has assessed the child in collaboration with allocated staff</li> </ul> <p>2. In the absence of medical staff, the shift coordinator can initiate observations followed by a request for the on-call psychiatrist to review the child as soon as practicable.</p> | <p>These factors must be taken into account when determining the level of clinical observations:</p> <ul style="list-style-type: none"> <li>• The severity of the patient's illness;</li> <li>• Their level of insight;</li> <li>• Expressed ability to adhere to management plan;</li> <li>• Degree of impulsivity; and</li> <li>• Presence of active suicidal ideation.</li> </ul> <ul style="list-style-type: none"> <li>• Refer to <a href="#">Appendix 1: Increased Observation Levels</a>.</li> </ul> |

### Implementing clinical observation levels

| Steps                                                                                                                                                                                                                                                                                                                                                                                                                            | Additional Information                                                                                                                                                                                                                                                                                                                                                                                                                         |
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| <p>For level one (1) and (2) special and close observation: Sequential step-by-step instructions that must be followed</p> <ol style="list-style-type: none"> <li>1. The CNM/CNS or shift co-ordinator is responsible for ensuring the clinician (and AIN, if relevant) allocated to the patient is sufficiently skilled to manage that patient.</li> <li>2. The psychiatrist (or medical delegate) must document the</li> </ol> | <p>When the clinician (and Assistant in Nursing (AIN), if relevant) hands over to another clinician (and AIN, if relevant) to continue the observation, it is the responsibility of both clinicians and AINs to ensure that an accurate account of mental state, risk and observation level is clearly communicated and understood. ISOBAR format must be followed.</p> <p>Refer to the <a href="#">CAMHS Clinical Handover Guideline</a>.</p> |

| Steps                                                                                                                                                                                                                                                                                                                                                            | Additional Information |
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| <p>observation level and what level of observation is required overnight.</p> <p>3. The psychiatrist (or medical delegate) must discuss level of care, any restriction and rationale with the patient.</p> <p>4. The psychiatrist or assigned nurse must inform next of kin of any change in levels of clinical observation as soon as practically possible.</p> |                        |

### Reviewing clinical observation levels for patient on increased levels of observation

| Steps                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    | Additional Information                                                                                                                                                                                                                                               |
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| <p>1. Clinical observation levels are to be reviewed by the treating team on every shift to approve ongoing use. The review must be informed from the outcome of the PRA.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | <p>Changes in clinical observation levels are to be documented by the psychiatrist (or medical delegate) in the patient's medical record immediately following the change. This must also be communicated verbally to the shift coordinator and allocated nurse.</p> |
| <p>2. Reviews must be held:</p> <ul style="list-style-type: none"> <li>• Morning shift: At the multidisciplinary team (MDT) meeting by the team, taking into consideration that least restrictive practice is always preferred. A consultant psychiatrist must have oversight of the decision to remove a special (Refer to the <a href="#">Multidisciplinary Team Clinical Reviews Policy</a>).</li> <li>• Afternoon shift: At the start of the shift at afternoon handover: CN/CNS to recommend review by medical team if required.</li> <li>• Night shift: At handover at 9:00 pm: CN/CNS to recommend review by medical team if required.</li> </ul> | <p>The CNS/CNM or duty CNM must be informed of any changes that impact on safe and appropriate staffing.</p>                                                                                                                                                         |
| <p>3. The psychiatrist (or medical delegate) must personally examine</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |                                                                                                                                                                                                                                                                      |

| Steps                                                                                                                                                                                                                                                                           | Additional Information |
|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------|
| <p>the child to assess risk, to inform decision making, for determination of the clinical observation level at a minimum of every 24 hours.</p> <p>4. Nursing staff must not reduce the level of observation in place.</p>                                                      |                        |
| <p>5. If nursing staff assess that current risk requires an increased level of observation, nurse initiated observations may be commenced and the psychiatrist or on-call registrar under the supervision of the on call psychiatrist will be notified as soon as possible.</p> |                        |

### Documentation required to assess clinical observation levels

| Steps                                                                                                                                                                                                                                                                                                                   | Additional Information                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <p>1. All observations must be recorded.</p> <p>2. The level of clinical observation must be documented in the:</p> <ul style="list-style-type: none"> <li>• Patient's Management Plan following assessment on admission; and</li> <li>• Medical record on the patient progress notes at each nursing shift.</li> </ul> | <p>In all cases, the reason for the assigned clinical observation level must be clearly stated, as well as:</p> <ul style="list-style-type: none"> <li>• The level of clinical observation</li> <li>• The frequency of clinical observation (for clinical observation level 3)</li> <li>• Whether the clinical observation level changes overnight.</li> </ul> <p>Observation checks and frequency of observations are recorded on the Visual Observations Chart.</p> <p>All documentation must be completed by a clinician not an Assistant in Nursing (AIN).</p> |

| References and related external legislation, policies, and guidelines                            |
|--------------------------------------------------------------------------------------------------|
| WALW <a href="#">Mental Health Act 2014</a>                                                      |
| WALW <a href="#">Mental Health Regulations 2015</a>                                              |
| <a href="#">The NSQHS Standards   Australian Commission on Safety and Quality in Health Care</a> |
| Related CAHS internal policies, procedures and guidelines                                        |
| <a href="#">CAMHS Clinical Handover Guideline</a>                                                |
| <a href="#">Multidisciplinary Team Clinical Reviews Policy</a>                                   |
| <a href="#">CAMHS Ward 5A Physical Restraint Policy</a>                                          |
| <a href="#">CAMHS Risk Assessment and Management Policy</a>                                      |
| <a href="#">CAMHS Ward 5A Seclusion Policy</a>                                                   |
| PCH <a href="#">Nursing Care: Increased Level of Observation and/ or Supervision</a>             |
| Useful resources (including related forms)                                                       |
| Visual Observations chart for patients at risk, MR816.01                                         |
| CAHS Risk Assessment and Management Plan (RAMP)                                                  |

## Increased Clinical Observation Levels in CAMHS Inpatient Services

This document can be made available in alternative formats on request.

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| Printed or personally saved electronic copies of this document are considered uncontrolled                                                                                                                                                                                                                                                                                       |                                                                                                                                                |                    |            |
| <div><div><h1>Healthy kids, healthy communities</h1><div><div>Compassion</div><div>Excellence</div><div>Collaboration</div><div>Accountability</div><div>Equity</div><div>Respect</div></div><div>Neonatology   Community Health   Mental Health   Perth Children’s Hospital</div></div></div> |                                                                                                                                                |                    |            |

## Appendix 1: Increased Observation Levels

|                                | Level 1 - Special                                                                                                                         | Level 2 – Close Observation                                                                                                                                                                                                                                                                       | Level 3 – Frequent Observation                                                                                                                                           | Level 4 – General Observation                                                                                 |
|--------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------|
| <b>Circumstance</b>            | Child is considered to be at very high immediate risk of suicide, self-harm; or other factors as deemed appropriate by the treating team. | Child needs constant but less intrusive monitoring. Considered to have risk of aggression, absconding, self-harm or other factors, as deemed appropriate by the treating team.                                                                                                                    | Child requires increase in nursing observations due to moderate risk of aggression, absconding and self-harm or other reason as deemed appropriate by the treating team. | The child requires general observation which is the minimum acceptable level of observation for all patients. |
| <b>Staff Allocation</b>        | One to one, in the same room as child at all times.                                                                                       | One-to-one.                                                                                                                                                                                                                                                                                       | No increased staffing level unless multiple patients on level 3 observations.                                                                                            | No increased staffing level.                                                                                  |
| <b>Visualisation</b>           | Within arm's length and eyesight at all times. Must be able to visualise the child's head and neck at all times                           | Within eyesight at all times. Must be able to visualise the child's head and neck at all times                                                                                                                                                                                                    | Child must be sighted every 15 or 30 minutes as recorded by the psychiatrist or on-call registrar under the supervision of the on-call psychiatrist.                     | Child must be sighted every 60 minutes.                                                                       |
| <b>Bathroom</b>                | Within arm's length and eyesight in bathroom and toilet.                                                                                  | Within eyesight at all times. Bathroom access can be varied as determined and documented by the psychiatrist or the on-call registrar under the supervision of the on-call psychiatrist to allow the child to use the bathroom and toilet alone but must be close enough for verbal conversation. | Child may use the bathroom and toilet alone.                                                                                                                             | Child may use the bathroom and toilet alone.                                                                  |
| <b>School Attendance/Leave</b> | As determined by treating team.                                                                                                           |                                                                                                                                                                                                                                                                                                   |                                                                                                                                                                          |                                                                                                               |
| <b>Documentation</b>           | Current mental state and location recorded on the Visual Observations Chart at least every 15 minutes.                                    | Current mental state and location recorded on the Visual Observations Chart at least every 15 minutes.                                                                                                                                                                                            | Current mental state and location recorded on the Visual Observations Chart at least every 15 or 30 minutes.                                                             | Current mental state and location recorded on the Visual Observations Chart at least every 60 minutes.        |